NCCP 00232 · VERSION 5

Your treatment, explained

Vinorelbine IV · non-small-cell lung cancer

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · smpc

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- The phases and how they connect
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- The treatment being discussed
- Checks before and during treatment
- Support and safety at home
- Supportive medicines
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

How the medicines work

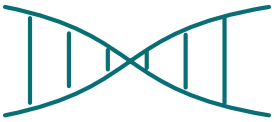
Vinorelbine chemotherapy can help control non-small-cell lung cancer and its symptoms. The team weighs possible benefit against side

effects, previous treatments and your priorities, and reviews whether to continue.

VINCA ALKALOID CHEMOTHERAPY

Vinorelbine

IV infusion



Disrupts the machinery of cell division

Vinorelbine disrupts the tiny structures a cell needs to divide. This can stop cancer cells multiplying. Healthy blood-forming cells, gut cells and nerves can also be affected.

Binds cell-division structures → Microtubules cannot function normally → Cell division is disrupted

What to watch for

Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

The phases and how they connect

21-DAY REFERENCE

Vinorelbine IV · non-small-cell lung cancer

21-day interval

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

These are the medicines in this exact schedule. Dose holds, missed tablets and omitted visits need instructions from your treating team.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Milestones are not equal time intervals. Your team books and reviews tests. Check blood count, kidney and liver function before each cycle, and blood counts on the day of every administration including Day 8. Review neuropathy, bowel symptoms and infection each cycle; abnormal results may postpone or change treatment.

Check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Blood / kidney / liver	Baseline blood and organ tests	Day 1 and Day 8 blood counts	Counts before every dose; organ tests each cycle	Review recovery and current dose	Follow unresolved abnormalities
Symptoms / medicines	Bowel / nerve / infection review	Report early symptoms and support problems	Review all medicines and side effects	Do not restart or escalate yourself	Agree continuation and response assessment
Special schedule check	Confirm Day 1 and Day 8 visits	Check Day 8 clearance	Continue 21-day pattern	Keep previous dose / nadir history	Personal plan; no booked dates implied

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00232 v5 · official regimen · HPRA Navelbine IV SmPC, 8 January 2024

THE WHOLE TREATMENT JOURNEY

Vinorelbine IV · non-small-cell lung cancer: treatment and checks

Non-small-cell lung cancer · IV monotherapy

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Treatment plan NCCP 00232	Confirm lung monotherapy plan and current dose	Days 1 and 8 each 21-day cycle	Continue while helpful and tolerated	Review dose and recovery before restart	Agree continuation / next treatment
Blood / kidney / liver NCCP 00232 · SMPC	Baseline blood and organ tests	Day 1 and Day 8 blood counts	Counts before every dose; organ tests each cycle	Review recovery and current dose	Follow unresolved abnormalities
Symptoms / medicines NCCP 00232 · SMPC	Bowel / nerve / infection review	Report early symptoms and support problems	Review all medicines and side effects	Do not restart or escalate yourself	Agree continuation and response assessment
Special schedule check NCCP 00232 · SMPC	Confirm Day 1 and Day 8 visits	Check Day 8 clearance	Continue 21-day pattern	Keep previous dose / nadir history	Personal plan; no booked dates implied

What each check is for

1

Treatment and dose review

Before every administration

Check blood count, kidney and liver function before each cycle, and blood counts on the day of every administration including Day 8. Review neuropathy, bowel symptoms and infection each cycle; abnormal results may postpone or change treatment.

Ask / act: The treating team confirms results, dose and suitability; this map does not authorise treatment.

Sources: NCCP 00232 · SMPC

2

Support and symptoms

Throughout treatment, including days at home

Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

Ask / act: Keep a written medicine plan and urgent-contact route.

Sources: NCCP 00232 · ORAL-ANTIEMETIC · NCIS

3

Response and continuation

At agreed clinical review points

Vinorelbine chemotherapy can help control non-small-cell lung cancer and its symptoms. The team weighs possible benefit against side effects, previous treatments and your priorities, and reviews whether to continue.

Ask / act: Confirm the current plan with the team.

Sources: NCCP 00232 · TRIAL

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?

- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

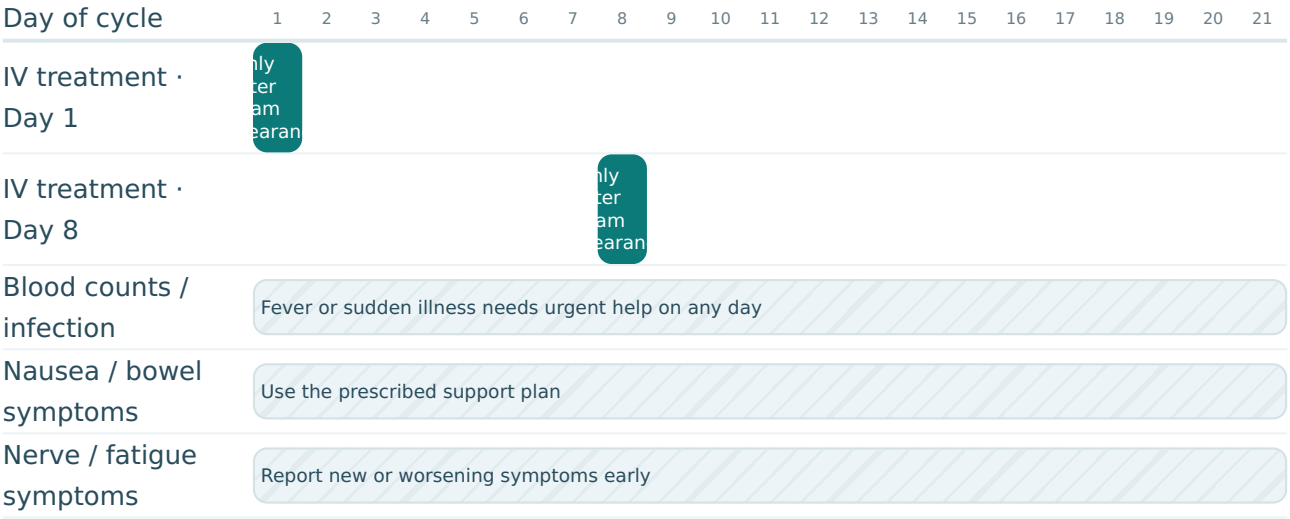
Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.29

Vinorelbine IV · non-small-cell lung cancer

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.



Treatment and recovery: 30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

Tests and decisions: Check blood count, kidney and liver function before each cycle, and blood counts on the day of every administration including Day 8. Review neuropathy, bowel symptoms and infection each cycle; abnormal results may postpone or change treatment.

> **Why these bands stay broad:** This is a relative reference, not an appointment calendar. Treatment days can change after clinical review. Symptom bands do not predict when an effect will occur. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00232 v5 · official regimen ↗

There are no guaranteed “safe days”. A day without treatment is not automatically a safe day. Use the emergency contact when needed; do not move or restart doses yourself.

Read the whole cycle as text

Day 1–21: One reference cycle

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation. Follow the written plan from your treatment team; no dates are entered here.

PRACTICAL TREATMENT GUIDE

Your practical treatment plan

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.



STEP 1

Visits and tests

Two infusion visits per 21-day cycle, with blood tests before doses. Peripheral cannula or existing access according to the treating service; a port is not automatically required.



STEP 2

Treatment preparation

Wear clothing allowing IV-site observation; report discomfort immediately.



STEP 3

At home

No home vinorelbine capsules are part of this regimen. Keep an up-to-date medication list and emergency contact.

Individual instructions

NCCP specifies 50 mL sodium chloride 0.9% over 15 minutes followed by 250 mL saline. Current Navelbine SmPC says 6–10 minutes: product/local-pharmacy reconciliation is required. Do not promise a total visit length; preparation, tests and observation add time.

Open practical treatment guide · **Printable practical guide**

Sources: NCCP 00232 v5 · official regimen · HPRA Navelbine IV SmPC, 8 January 2024. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Sickness plan

No routine preventive anti-sickness medicine is usually required for this IV regimen. Ask for a rescue plan and report uncontrolled symptoms.

2 Bowel care

Agree a constipation plan, particularly if using opioids or ondansetron. Severe abdominal pain, swelling, repeated vomiting or inability to pass stool/gas needs urgent assessment; do not simply add laxatives.

3 Medicines and vaccines

Check every new prescription, supplement and vaccine with the team; avoid grapefruit. Live vaccines and interacting medicines need review.

Sources: nccp · oral-antiemetic · ncis

TREATMENT REFERENCE · 01

The treatment being discussed

Vinorelbine chemotherapy can help control non-small-cell lung cancer and its symptoms. The team weighs possible benefit against side

effects, previous treatments and your priorities, and reviews whether to continue.

Lung treatment context

NCCP 00232b: non-small-cell lung cancer. This guide covers vinorelbine monotherapy. Platinum combinations, adjuvant treatment and radiotherapy combinations need their own complete plans; this guide does not authorise them.

Sources: nccp

Schedule and duration

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

Sources: nccp

How treatment is given

NCCP specifies 50 mL sodium chloride 0.9% over 15 minutes followed by 250 mL saline. Current Navelbine SmPC says 6–10 minutes: product/local-pharmacy reconciliation is required. Do not promise a total visit length; preparation, tests and observation add time.

Sources: nccp

TREATMENT REFERENCE · 02

Checks before and during treatment

Every dose depends on the reviewed treatment plan and results.

Blood and symptom checks

Check blood count, kidney and liver function before each cycle, and blood counts on the day of every administration including Day 8. Review neuropathy, bowel symptoms and infection each cycle; abnormal results may postpone or change treatment.

Sources: nccp

Heart and response checks

Review cardiovascular history and new symptoms. These monotherapy NCCP protocols do not mandate the serial LVEF/echo schedule used for anthracyclines or trastuzumab; arrange cardiac tests when clinically indicated.

Sources: nccp

Fertility, pregnancy and breastfeeding

Discuss the disease-control aim, likely benefit and uncertainty, other lung-cancer treatments or symptom-focused care, and the burden of visits and tests. Review low blood counts and infection, nausea/vomiting, fatigue, constipation/ileus, nerve symptoms, hair changes and uncommon serious lung/heart/allergic effects. Discuss IV-site injury and the different IV and capsule doses; the product-specific infusion plan needs pharmacy confirmation. Discuss fertility preservation and pregnancy planning. Do not breastfeed. Confirm the exact product contraception advice: HPRA recommends contraception during treatment and for 7 months afterwards for women, and 4 months for men.

Sources: smpc

TREATMENT REFERENCE · 03

Support and safety at home

Keep the written medicine and urgent-contact plan.

Sickness plan

No routine preventive anti-sickness medicine is usually required for this IV regimen. Ask for a rescue plan and report uncontrolled symptoms.

Sources: nccp

Bowel care

Agree a constipation plan, particularly if using opioids or ondansetron. Severe abdominal pain, swelling, repeated vomiting or inability to pass stool/gas needs urgent assessment; do not simply add laxatives.

Sources: nccp

Medicines and vaccines

Check every new prescription, supplement and vaccine with the team; avoid grapefruit. Live vaccines and interacting medicines need review.

Sources: nccp

Urgent contact

Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

Sources: nccp

Why this treatment may be chosen

Studies support vinorelbine as an option in selected people with advanced non-small-cell lung cancer. The strength and type of evidence differ for the IV and capsule schedules.

The team considers tumour tests, previous treatment, health, symptoms and your preferences. Results from a group cannot predict your own benefit.

Sources: nccp · trial

PREPARE FOR THE CONVERSATION

Questions worth asking

Can IV and oral vinorelbine be exchanged?

For IV use only; intrathecal administration may be fatal. Oral and IV vinorelbine are not interchangeable mg-for-mg. Stop infusion and tell staff immediately about pain, burning, swelling or leakage at the IV site; extravasation can damage tissue.

How long will treatment continue?

Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

Do I need regular heart scans?

Review cardiovascular history and new symptoms. These monotherapy NCCP protocols do not mandate the serial LVEF/echo schedule used for anthracyclines or trastuzumab; arrange cardiac tests when clinically indicated.

How long is an infusion visit?

The source describes a short infusion followed by a saline flush. Your unit confirms the product-specific infusion instructions and complete visit length, including blood results, preparation and observation.

> **Try explaining the plan back:** Which treatment days and doses are prescribed, which blood tests must happen first, how do you take the support medicines, and which symptoms need urgent help?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00232 v5. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00232 v5 · official regimen
- HPRA Navelbine IV SmPC, 8 January 2024
- NCCP antiemetic guidance v6
- NCCP NCIS antiemetic medicines v8
- ELVIS Group, JNCI 1999: IV vinorelbine versus supportive care in older adults with advanced NSCLC

SACTCheck · Developed by Dr Paul O’Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6567/232_v5_Vinorelbine_IV.pdf

https://assets.hpra.ie/products/Human/26035/Licence_PA0329-011-003_08012024110935.pdf

https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://pubmed.ncbi.nlm.nih.gov/9890172/